



POLICY

Psychotropic Medication – Monitoring Adverse Physical Health Effects

Scope (Staff):	Clinical Staff
Scope (Area):	All CAMHS services

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations.

Read the full statement here:

[CAHS Child Safe Organisation Commitment Statement](#)

This document should be read in conjunction with the [CAHS High Risk Medicines Policy](#) and [CAMHS Psychotropic Medication Physical Health Monitoring Handbook](#).

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Aim

To inform staff on optimal monitoring for adverse drug effects (ADE) of psychotropic medication on the physical health of patients. It applies to all children who are being prescribed psychotropic medication within all CAMHS services.

Risk

The adverse effects impacting on physical health and wellbeing will not be responded to in a timely manner.

Background

The use of psychotropic medications must be monitored closely to help ensure that young people are treated safely and effectively. Children are more vulnerable than adults to developing several antipsychotic induced ADE and medications are often administered for long time periods potentially affecting growth, and development.

Polypharmacy is a major risk factor for exposure to potential drug-drug interactions. Medications used concurrently have a potentially multiplicative increase in the number of ADE, especially for specific drug classes causing sedation or QT-prolongation.

CAMHS strives to support clinicians in minimising exposure to harmful drug-drug interactions and maintain greater vigilance during the monitoring for ADEs, while paying attention to overall health outcomes.

Definitions

Child: An infant, child, adolescent, or young person under the age of 18 years.

Family: The person(s) who has legal responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (MHA s 4).

Intra-class Polypharmacy: More than two medications prescribed at the same time within the same class, other than for cross-tapering purposes.

Inter-class Polypharmacy: More than three medications prescribed at the same time from different classes of medications for overall treatment of mental health disorders.

Prescriber: For the purposes of this policy, a prescriber is a medical practitioner responsible for the prescription and medical oversight of psychotropic medication.

Key points

- Commonly used psychotropic medication combinations include the following:
 - Medication combinations used to treat multiple disorders in the same patient.
 - Medication combinations offering treatment advantages for a single disorder.
 - Medication combinations to address ADE of an effective agent.
- The rationale and justification for the combined use of polypharmacy (intra-class and inter-class) must be documented in the medical record.

- In reviewing medication regimens the prescriber will consider the wishes of the patient and their family and the patient's best interest and health outcomes, while remaining generally prudent and cautious in prescribing. Prescribers are to be guided by CAHS approved primary paediatric references and the Paediatric Statewide Medicines Formulary.
- Clinical staff are required to have an understanding of medication adverse effects potentially impacting physical health prior to prescribing and administration.

Procedure

Consent

CAMHS must ensure that children and families agree with and are actively involved in the decision-making process and have adequate information about their management. This can be achieved through informed consent, which constitutes the process of obtaining patient's consent for a health practitioner to provide healthcare treatment.

Consent processes are to be in accordance with the [MP 0175/22 Consent to Treatment Policy](#) and [CAMHS Consent Guideline](#).

The informed consent process must include:

- Discussion about the patient's clinical condition, and treatment options, including evidence for use of medication.
- Discussion about strengths/benefits and challenges/risks of each treatment option, including short- and long-term physical health effects.
- Providing written and verbal information to support the patient and family's understanding of their clinical condition and available treatment options. Information provided needs to be in a language and manner suitable for the patient's cultural, language and communication needs.
- Opportunities provided to the patient and/or family to raise concerns and ask questions about the treatment.
- Obtaining patient consent for a specific treatment option and documenting the consent and any relevant discussion.

The CAHS Consent for Medication Use Form (MR505.00/MR859.01) must be used to document consent.

Prescribing

When prescribing any medication, clinicians must ensure that they are aware of all other relevant policies (including local site policies) and operational circulars/directives that relate to the medication being prescribed and to read and adhere to the available prescribing information.

All patients receiving psychotropic medication require a baseline physical health assessment prior to commencement of medication. Reasonable clinical judgment, supported by available assessment information, must guide the prescription of psychotropic medications.

Physical health assessments are to include:

- Baseline assessments/investigations
- Co-morbid use of alcohol and other drugs (including nicotine and non-prescribed medications).
- Lifestyle factors, such as diet and exercise.
- Co-morbid/risk of medical conditions (including metabolic disease).

All off-label medications should be prescribed in accordance with the [CAHS Use of Medications for Other Than Manufacturer's Approved Indications Policy](#) and the [CAMHS Off-Label Psychotropic Prescribing Guide for CAMHS Clinicians Guideline](#).

Under section 304 of the MHA 2014⁷ the provision of off-label treatment to any child who is an involuntary patient must be reported to the Chief Psychiatrist.

Prescribing of medications where care is shared between CAMHS and outside services or alternates between acute, community and specialised settings within CAMHS, should be decided following a discussion between a primary prescriber and other prescribers involved in providing care for the patient.

Sharing of information regarding medication must be in accordance with the [CAHS Confidentiality, Disclosure and Transmission of Health Information Policy](#), the [CAMHS Clinical Handover Guideline](#) and the [Shared Care in CAMHS Guideline](#).

Refer to the following resources for further information related to prescribing:

- [MP 0131/20 High Risk Medication Policy](#)
- [MP 0078/18 Medication Chart Policy](#)
- [CAMHS Physical Health Assessment and Monitoring Policy](#)
- [Australian Medicines Handbook](#) and [AusDI](#)
- [Stimulant Medicines WA Health](#)

Flexible Prescribing

There are some patients who occasionally require titrating the dose of various medications against symptoms (PRN medications and introduction of new medications). Patients and/or their family should be advised about the appropriate monitoring of mental state and signs and symptoms of ADE.

Patients and families should be informed to follow the medication treatment plan, including titration of medication against their symptoms and when to escalate concerns. Prescribers should ensure that follow up appointments are scheduled at appropriate monitoring intervals.

The following are guidelines for the monitoring and prescription of PRN medications:

- If the prescriber believes that PRN medications will be necessary, they must prescribe a sufficient number to be dispensed by the pharmacy and advise the patient and family accordingly.

- If the patient requires a significant increase or decrease in the medication regime, this must be clearly documented in the medical record by the prescriber with a maximum daily dose stated.

Where there have been major changes in the medication that are significantly different from the medication regime prescribed, the patient must be assessed by the prescriber within 72 hours.

Monitoring

Routine

All patients receiving psychotropic medication require regular monitoring, including:

- Repeat assessments and investigations at regular intervals throughout the duration of treatment
- Ongoing monitoring for ADE, including extrapyramidal side effects

All assessment results must be communicated to other relevant care providers, including referral for further management of emerging conditions. Two-way communication with other care providers is to be encouraged.

Responsibility for monitoring patients receiving psychotropic medication and communicating assessment results lies with the primary prescriber, or a delegate. Any delegation of responsibility for prescription must be clearly documented.

All medication use and review must be fully documented in the medical record.

For monitoring ADE of psychotropic medication, refer to the Psychotropic Physical Health Monitoring chart, contained within the [CAMHS Psychotropic Medication Physical Health Monitoring Handbook](#), [CAMHS Physical Examination Form](#), and [Abnormal Involuntary Movement Scale \(AIMS\)](#).

Specific Medications

Patients may require more specific assessments, investigations or monitoring in addition to routine monitoring if they are prescribed:

- Antipsychotic medication
- Stimulant and non stimulant medication for ADHD
- Antidepressants
- Mood stabilisers
- Sedatives

Refer to the [CAMHS Psychotropic Medication Physical Health Monitoring Handbook](#) and relevant PCH Medication Monograph for indicated investigations.

All patients prescribed psychotropic medications will:

- Be monitored for metabolic, cardiovascular and neurological ADE

- Receive a medication review of all medications, including non-psychotropic medications and supplements to avoid potential drug-drug interactions.
- Be assessed for potential risk factors to the prolongation of QTc interval
- Receive baseline and annual ECGs to monitor for prolonged QTc interval

Depending on the specific psychotropic medication(s) prescribed, parameters must be monitored according to national guidelines taking into account individualised factors.

The [Antipsychotic Physical Health Monitoring Form](#) has been developed to support clinicians in the safe use of antipsychotic medication. This includes risk factors identified for cardiac and metabolic disorders. This form must be initiated prior to commencing antipsychotic medication.

Presence of abnormal results should be noted on the physical examination form. Cardiology review is to be sought for prolonged QTc interval where clinically indicated.

Further information is available through the following resources:

Medication	Resource
Clozapine	PCH Clozapine Medication Monograph Guidelines for the Safe and Quality Use of Clozapine Therapy in the WA Health System
Droperidol	PCH Droperidol Medication Monograph
Lithium	PCH Lithium Medication Monograph
Olanzapine pamoate	PCH Olanzapine Pamoate Depot Injection Medication Monograph.
Zuclopentixol acetate	PCH Zuclopentixol Acetate Medication Monograph.

Compliance Monitoring

Compliance is monitored through the Antipsychotic Monitoring Survey by the CAMHS Medication Safety Committee in accord with the CAMHS Audit Schedule as approved by the CAMHS Executive Committee.

Clinical incidents notified in CIMS under this policy are monitored by CAMHS Safety and Quality with oversight by the CAMHS Executive Committee.

Notifiable incidents (as defined by the [Mental Health Act 2014](#) (WA)), must be reported to the Chief Psychiatrist in accordance with the [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#).

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14. Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry. 13th Edition. West Sussex. Wiley Blackwell; 2018.

Related policies, procedures and guidelines
CAHS High Risk Medicines Policy
CAHS Medication Safety Policy
CAHS Use of Medications for Other Than Manufacturer's Approved Indications
CAMHS Agitation and Arousal Management for Mental Health Inpatients Policy
CAMHS Consent Guideline
CAMHS Off-Label Psychotropic Prescribing Guide for CAMHS Clinicians Guideline
CAMHS Physical Health Assessment and Monitoring Policy
CAMHS Shared Care in CAMHS Guideline
Chief Psychiatrist's Standards for Clinical Care – Physical Health
PCH Agitation and Arousal Medication Management Policy
PCH Clozapine Medication Monograph
PCH Doperidol Medication Monograph
PCH Lithium Medication Monograph
PCH Olanzapine Pamoate Depot Injection Medication Monograph.
PCH Zuclopenthixol Acetate Medication Monograph.
WA Health Consent to Treatment Policy - MP 0175/22
WA Health Guidelines for the Safe and Quality Use of Clozapine Therapy in the WA Health System

[WA Health High Risk Medication Policy - MP 0131/20](#)

[WA Health Medication Chart Policy - MP 0078/18](#)

Useful resources (including related forms)

CAMHS Antipsychotic Physical Health Monitoring Form (MR505.01)

CAMHS Consent for Medication Use Form (MR505.00/MR859.01)

[CAMHS Medication Safety Toolbox](#)

[CAMHS Psychotropic Medication Physical Health Monitoring Handbook](#)

[Abnormal Involuntary Movement Scale \(AIMS\)](#)

This document can be made available in alternative formats on request.

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